Chronic Kidney Disease: Chapter 1

CKD in the General Population

Data Notice

Please note: The data and analyses presented in this chapter are identical to those published in the 2025 Annual Data Report.

Highlights

- **Scope of CKD:** Fourteen percent of adults in the U.S. have chronic kidney disease (CKD), defined as estimated glomerular filtration rate <60 ml/min/1.73m^2, albuminuria, or both (those in yellow, orange, or red in Table 1.1) based on NHANES data from 2017-March 2020.
- **Racial/ethnic variation:** The percentage of people with CKD was highest among Black adults (18.8%) and lowest among Hispanic adults (12.0%) (Figure 1.1).
- **Diabetes:** The percentage of adults in the U.S. with diabetes rose, reaching 35.6% in those with CKD compared with 9.5% in those without (Figure 1.6).
- **Association with insurance coverage:** The rate of CKD was twice as high among people with Medicare coverage as among those with Medicaid or private insurance (Table 1.2)

Introduction

In May 2025, chronic kidney disease (CKD) was recognized as a global health priority by the World Health Organization (WHO). As part of this effort, the WHO resolution urges Member States to "strengthen and integrate the monitoring of kidney disease burden, access to care, quality of care and morbidity and mortality outcomes into national health information systems, to inform policy decisions and guide research." The epidemiology of CKD has long been a priority of the USRDS Annual Data Report (ADR) but is challenging to study in a nation with a complex and diverse healthcare system. In the U.S, there are many different insurers, and yet many people remain uninsured or underinsured. As such, there is no universal registry for CKD, unlike for treated end-stage kidney disease, or ESRD.

To accomplish its mission, the ADR relies heavily on data from the National Health and Nutrition Examination Survey (NHANES). However, CKD is harder to study epidemiologically than some other conditions because it is diagnosed on the basis of "abnormalities of kidney structure or function present for >3 months with implications for health" (Stevens et al., 2024). However, NHANES measures kidney function on only a single occasion and people cannot be followed over time. Therefore, the ADR must estimate CKD stages based on the single estimates of glomerular filtration rate (GFR) and albuminuria available in NHANES, an approach that will invariably overestimate the prevalence of CKD compared with the clinical practice guideline requirement for persistently abnormal values for three or more months.

NHANES includes data derived from interviews, physical examinations, and laboratory testing, but it is important to note that diabetes mellitus and hypertension (key conditions that often coexist with CKD) are defined using participant self-report, examination or laboratory testing, and/or medication use, whereas cardiovascular disease is based on self-report only as hospital records are not available. NHANES, which was curtailed in March 2020 due to the COVID-19 pandemic, is representative of the non-institutionalized U.S. population, with oversampling of certain subgroups to increase reliability and precision of health indicator estimates. In the 2026 ADR, we report NHANES results for 2017-March 2020 and examine long-term trends based on three prior four-year periods — 2005-2008, 2009-2012, and 2013-2016.

In the chapter, we present the prevalence of CKD by demographic characteristics and clinical conditions. We then examine the prevalence of low eGFR and albuminuria separately. Finally, we present the prevalence of diabetes by CKD status and show data on self-reported dietary and lifestyle/behavioral activities.

Table 1.1 Percentage and number of U.S. adults in the KDIGO CKD risk categories

Percentage | Number of Persons

(a) Percentage by eGFR and ACR, 2017-March, 2020

eGFR Categories	A1: Normal to mildly increased (ACR <30 mg/g)	A2: Moderately increased (ACR 30-299 mg/g)	A3: Severely increased (ACR ≥300 mg/g)	Total
G1: Normal or high (eGFR ≥90mL/min/1.73m²)	59.8	5.0	0.68	65.5
G2: Mildly decreased (eGFR 60-89 mL/min/1.73m²)	26.2	2.4	0.35	28.9
G3a: Mildly to moderately decreased (eGFR 45-59 mL/min/1.73m²)	3.1	0.79	0.12	4.0
G3b: Moderately to severely decreased (eGFR 30-44 mL/min/1.73m²)	0.61	0.32	0.18	1.1
G4: Severely decreased (eGFR 15-29 mL/min/1.73m²)	0.07	0.08	0.18	0.34
G5: Kidney failure (eGFR <15 mL/min/1.73m²)	0.00	0.02	0.13	0.15
Total	89.8	8.6	1.6	100

(b) Trends in percentage in KDIGO CKD risk categories

KDIGO CKD Risk Categories	2005-2008	2009-2012	2013-2016	2017-March, 2020
Low risk	86.7	87.5	86.1	86.0
Moderately high risk	9.6	9.0	10.2	10.5
High risk	2.4	2.3	2.5	2.4
Very high risk	1.3	1.2	1.3	1.1
Total	100.0	100.0	100.0	100.0

Key point

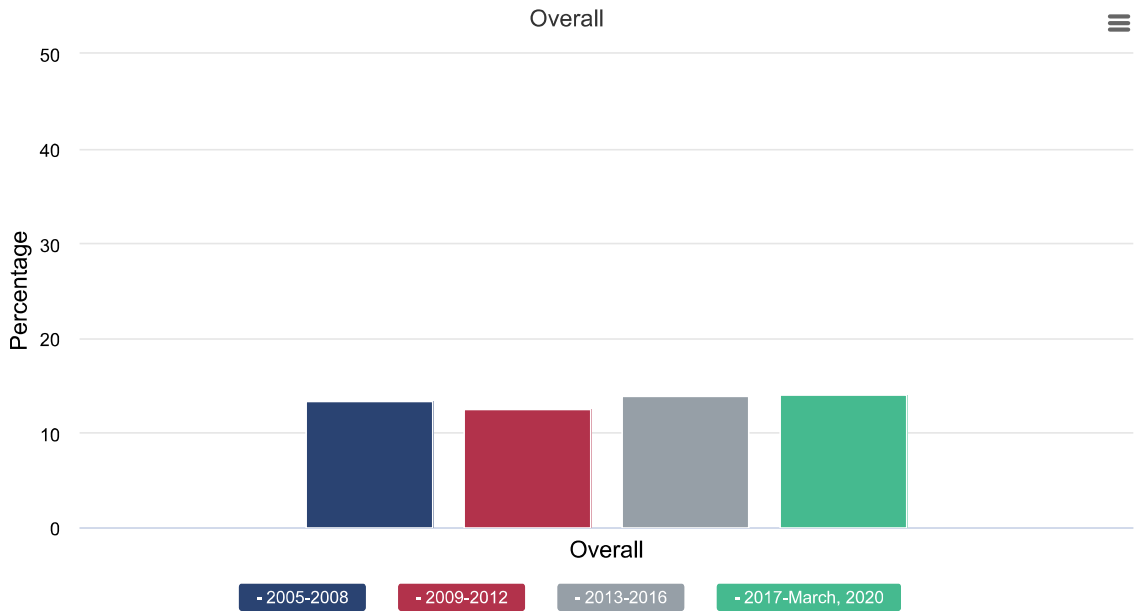
In NHANES data from 2017-March 2020, 14.0% of U.S. adults had a low eGFR, albuminuria, or both.

Additional context

There were 31.2 million people in 2017-March 2020 with a low eGFR, albuminuria, or both. Of these, 10.5% were moderate risk, 2.4% were high risk, and 1.1% were very high risk. Although the percentages changed relatively little over time, the growth of the U.S. population means that, in absolute terms, there was a 19.1% increase in the *number* of people with CKD between 2005-2008 and 2017-March 2020.

Figure 1.1 Percentage of U.S. adults with CKD

Overall | Age | Sex | Race/Ethnicity | Diabetes | Hypertension | Cardiovascular Disease | Obesity



Key point

The percentage of U.S. adults with CKD has been relatively stable since 2005-2008 and was 14.0% in 2017-March 2020.

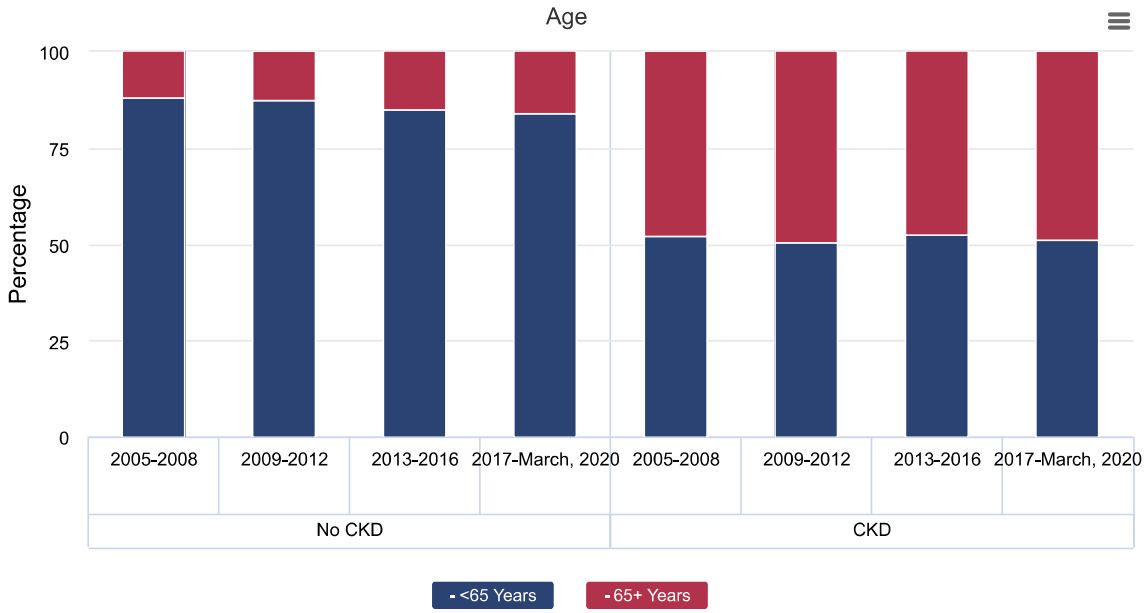
Additional context

From 2005-2008 to 2017-March 2020, the percentage of adults aged ≥65 years with CKD decreased steadily, from 38.0% in 2005-2008 to 33.2% in 2017-March 2020. A higher percentage of women (15.4% in 2017-March 2020) than men (12.6%) had CKD, a finding that has been consistent since 2005-2008. In the most recent survey period, CKD prevalence was highest among Black people (18.8%) and lowest among Hispanic people (12.0%).

The percentage of people with cardiovascular disease who had CKD decreased from 40.2% in 2005-2008 to 34.9% in 2017-March 2020. In contrast, the percentage of people with diabetes who had CKD decreased from 2005-2008 to 2013-2016 before increasing to 38.0% in 2017-March 2020.

Figure 1.2 Demographics of people with and without CKD

Age | Sex | Race/Ethnicity | Diabetes | Hypertension | Cardiovascular Disease | Obesity



Key point

Among people with CKD, the percentage who were aged ≥65 years did not change over time despite the aging of the non-CKD population.

Additional context

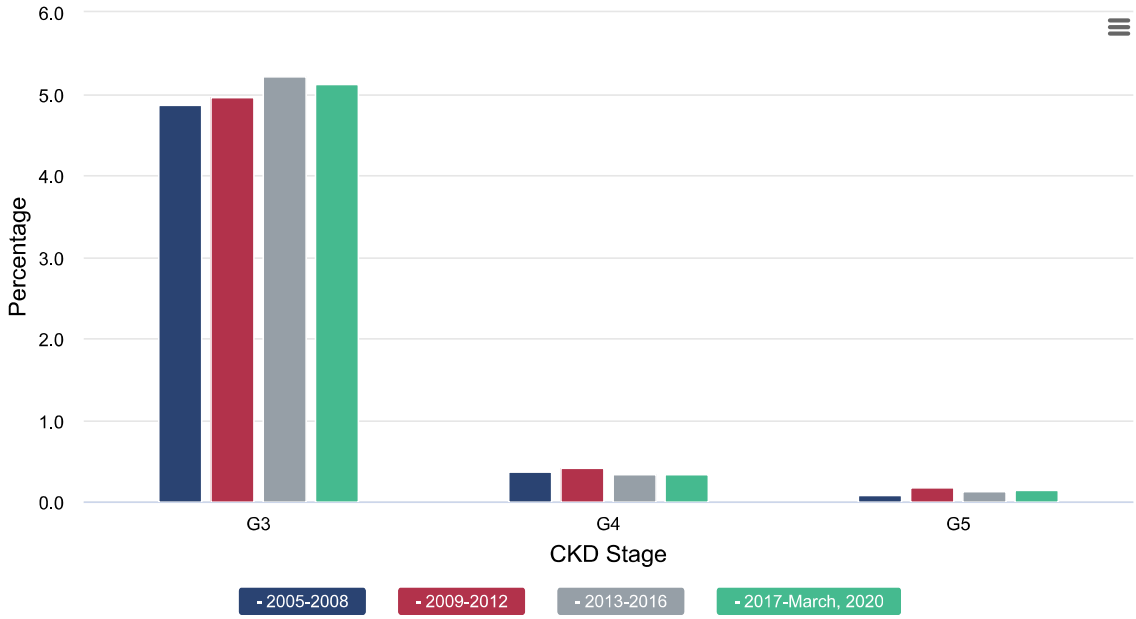
Although the percentage of people without CKD who were aged ≥65 years increased from 11.9% in 2005-2008 to 15.9% in 2017-March 2020, the percentage of people with CKD who were aged ≥65 years was virtually identical in 2005-2008 and 2017-March 2020.

Among people with CKD, women were relatively overrepresented in all time periods. Although the proportion of White people in both samples (CKD and non-CKD) decreased over time, there were no major changes in the proportion with CKD. Black people were overrepresented in the CKD groups, but the percentage did not change substantively over time. There was a steady increase in the percentage of people who self-identified as Hispanic, but the percentage who were Hispanic was consistently lower among those with CKD than among those without.

Although the percentage of people with CKD who had diabetes increased over time, from 27.3% in 2005-2008 to 35.6% in 2017-March 2020, this was also the case for people without CKD, in whom diabetes increased from 6.5% in 2005-2008 to 9.5% in 2017-March 2020. Obesity increased over time, and in all periods, the percentage of people with obesity was higher among those with CKD than among those without.

Figure 1.3 Stages of CKD in U.S. adults

Overall | Age | Sex | Race/Ethnicity



Key point

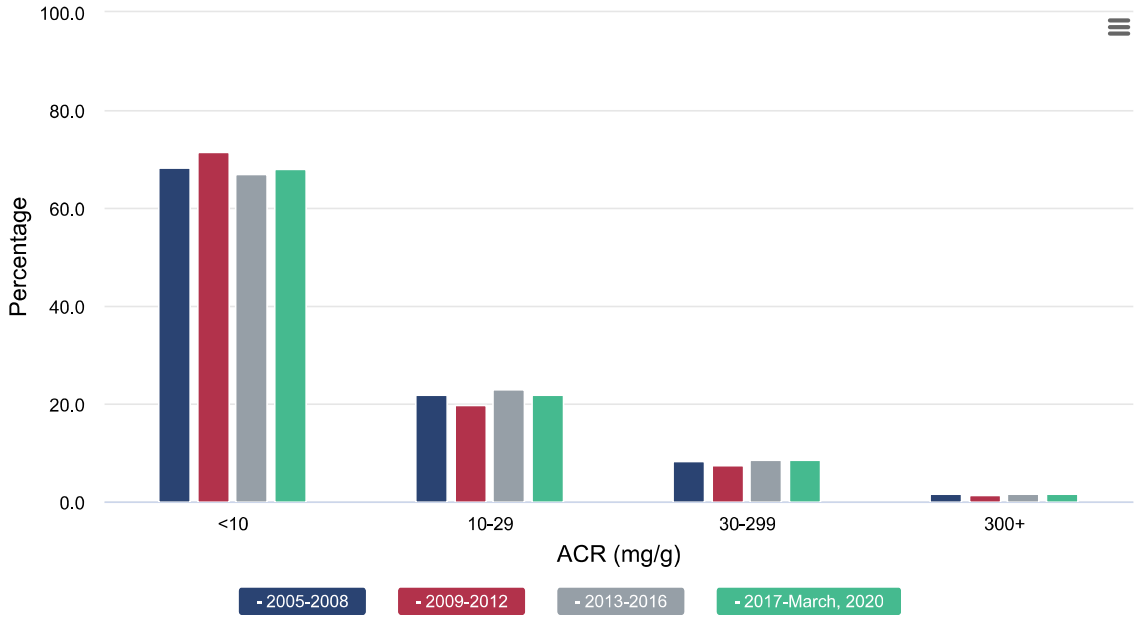
In 2017-March 2020, 5.1% of adults had stage 3, 0.3% stage 4, and 0.2% stage 5 CKD.

Additional context

CKD was much more common among people aged ≥65 years than among those aged <65 years. Between 2013-2016 and 2017-March 2020, the percentage of people aged <65 years with CKD stage 3 decreased from 1.6% to 1.2% but was virtually unchanged among those aged ≥65 years. Over this period, the prevalence of stages 3 and 4 CKD increased in Black people, was unchanged among White people, and decreased slightly in Hispanic people. In contrast, there was a decrease in stage 5 CKD among Black people but a slight increase among Hispanic people.

Figure 1.4 Urinary albumin to creatinine ratio in U.S. adults

Overall | Age | Sex | Race/Ethnicity



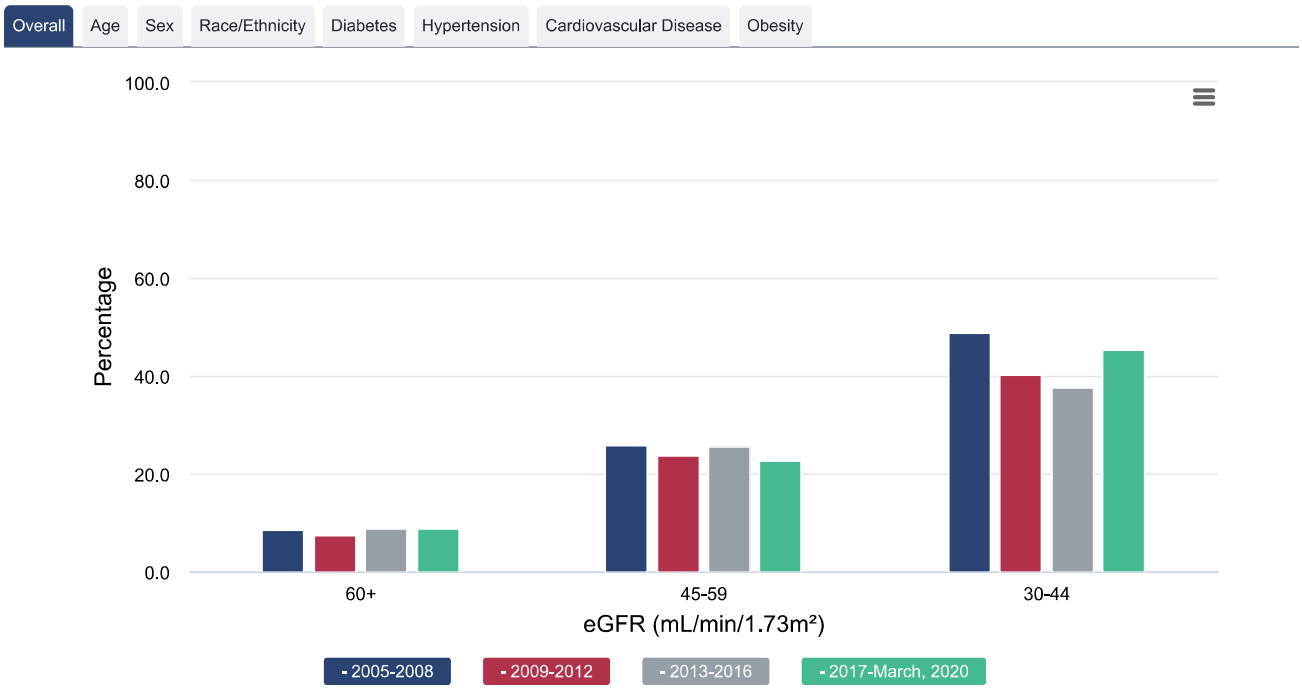
Key point

Overall, the percentage of people with ACR ≥30 mg/g was relatively stable between 2005-2008 (9.9%) and 2017-March 2020 (10.2%).

Additional context

In 2017-March 2020, the percentage of Black people with ACR ≥30 mg/g (13.5%) was higher than the percentage of White (9.2%) and Hispanic (10.9%) people with ACR ≥30 mg/g.

Figure 1.5 Percentage of U.S adults with urinary albumin to creatinine ratio ≥30 mg/g, by estimated glomerular filtration rate



Key point

Between the first and last period examined, the percentage of people with ACR ≥30 mg/g but eGFR ≥60 mL/min/1.73m^2 increased slightly from 8.4% to 8.9%.

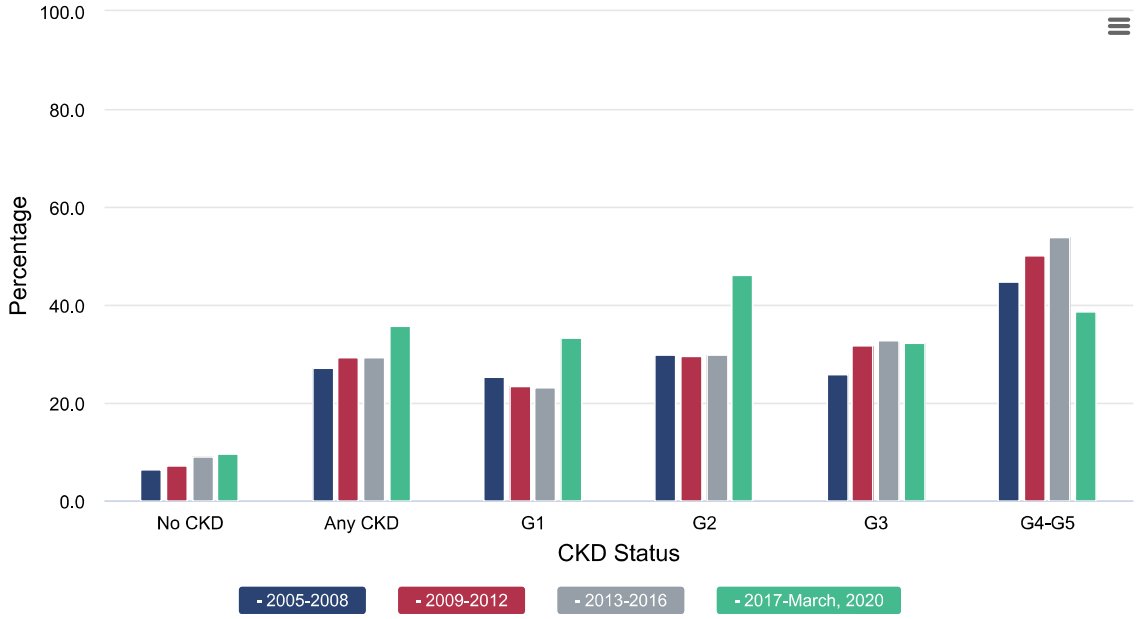
Additional context

Over the period examined, the percentage of people with ACR ≥30 mg/g decreased from 25.8% to 22.7% for those with eGFR 45-59 mL/min/1.73m^2 and decreased from 48.4% to 45.2% for those with eGFR 30-44 mL/min/1.73m^2.

In 2017-March 2020, ACR ≥30 mg/g was more common in Black people than in White people. For example, for eGFR 45-59 mL/min/1.73m^2, the percentage of Black people with ACR ≥30 mg/g was 30.0% versus 19.6% for White people.

In terms of comorbid conditions, among people with diabetes and eGFR 30-44 mL/min/1.73m^2, the percentage with ACR ≥30 mg/g increased from 52.3% in 2005-2008 to 61.4% in 2017-March 2020, but there were no clear changes in the percentage with ACR ≥30 mg/g when the eGFR was 45 mL/min/1.73m^2 and above.

Figure 1.6 Percentage of U.S. adults with diabetes mellitus, by CKD status



Key point

The percentage of people with diabetes increased markedly from 2005-2008 to 2017-March 2020 in those with and without CKD, reaching 9.5% among those without CKD (i.e., without albuminuria or eGFR <60 mL/min/1.73m^2) and 35.6% among those with CKD.

Additional context

The increase in diabetes among people with CKD occurred primarily among those with stages 1, 2, and 3 CKD: among people with stage 3 CKD, for example, the increase was from 25.9% to 32.3%, although almost all of the increase occurred between 2005-2008 and 2009-2012. Diabetes decreased among people with stage 4-5 CKD, from a peak of 53.8% in 2013-2016 to 38.6% in 2017-March 2020.

Table 1.2 Percentage of U.S. adults with CKD by insurance, income, and education level

	Overall (%)				Age ≥65 Years (%)			
Characteristic	2005-2008	2009-2012	2013-2016	2017-March, 2020	2005-2008	2009-2012	2013-2016	2017-March, 2020
Overall	13.3	12.5	13.9	14.0	38.0	36.1	34.1	33.2
Health insurance								
Not insured	8.9	8.7	10.9	10.8	37.3	38.6	46.1	42.7
Insured	14.3	13.5	14.5	14.5	38.0	36.0	33.9	33.1
Health insurance type								
Private	12.0	10.9	12.1	13.2	37.8	32.8	32.8	33.6
Medicare	36.2	35.6	32.2	31.5	38.6	37.0	33.5	33.3
Medicare and private	37.4	33.7	32.0	33.6	38.7	34.4	32.7	34.2
Medicaid	23.0	18.9	20.4	15.6	47.4	43.6	43.4	42.5
Other government	19.0	15.8	17.8	14.3	34.8	37.0	35.6	28.1
Family income to poverty ratio								
≥1: Not poverty	12.7	11.9	13.1	14.0	37.2	35.4	33.0	33.6
<1: Poverty	15.3	14.5	18.8	16.5	40.9	44.0	47.4	39.7
Education								
Not a high school graduate	19.7	19.0	19.1	20.4	45.9	46.0	43.3	39.6
High school graduate/GED	15.1	13.9	16.3	16.1	39.2	38.5	37.4	37.9
At least some college	10.4	10.3	11.9	12.0	31.9	30.3	30.2	29.5

Key point

Among uninsured people, the percentage with CKD increased from 8.9% to 10.8% between 2005-2008 and 2017-March 2020.

Additional context

The percentage of insured people with CKD in 2017-March 2020, at 14.5%, was relatively stable since 2005-2008. Among the subset of insured people aged ≥65 years, the percentage with CKD decreased from 38.0% to 33.1%.

The percentage of people insured by Medicare with CKD declined from 2005-2008 to 2017-March 2020, consistent with the decline noted among people aged >65 years, as shown in Figure 1.1. CKD also declined among those insured by Medicaid, possibly related to the expansion of this population in recent years (in which newly covered people may have represented a healthier group).

In 2017-March 2020, the overall percentage of people with CKD was more than twice as high among people insured by Medicare than among people insured by Medicaid, private insurance alone, other governmental (typically military) insurance, or no insurance.

The percentage with CKD was also higher among people with lower income and among those with lower education.

Table 1.3 Health risk behaviors by CKD status in U.S. adults

Adjusted | Unadjusted

	No CKD (%)				CKD (%)			
Characteristic	2005-2008	2009-2012	2013-2016	2017-March, 2020	2005-2008	2009-2012	2013-2016	2017-March, 2020
Sedentary behavior	--	24.0	23.9	20.8	--	33.6	28.4	26.2
Smoking status								
Never	52.8	56.4	57.3	57.8	52.3	52.9	50.1	56.6
Former	25.8	24.8	24.4	25.8	24.1	26.0	27.3	26.0
Current	21.3	18.8	18.3	16.4	23.3	20.9	22.5	17.1
Vaping status	--	--	1.9	3.0	--	--	2.3	3.6
Daily dietary intake								
Energy, ≥2000 kcal	47.9	49.4	48.8	49.7	42.6	45.5	46.5	46.4
Sodium, ≥2.3 grams	76.6	80.6	80.1	77.8	73.3	76.5	75.8	71.6
Potassium, <4.7 grams	95.3	94.9	95.6	95.6	95.0	97.0	96.0	97.9
	eGFR <60 mL/min/1.73m^2 (%)				ACR ≥30 mg/g (%)			
Characteristic	2005-2008	2009-2012	2013-2016	2017-March, 2020	2005-2008	2009-2012	2013-2016	2017-March, 2020
Sedentary behavior	--	37.2	36.9	27.6	--	35.2	28.8	27.0
Smoking status								
Never	52.9	45.6	64.4	61.2	50.9	52.1	48.2	56.8
Former	31.0	38.6	23.8	25.2	24.3	26.0	27.9	25.3
Current	15.3	15.1	11.6	13.6	24.4	21.8	23.9	17.7
Vaping status	--	--	0.6	0.1	--	--	2.4	4.0
Daily dietary intake								
Energy, ≥2000 kcal	34.0	37.3	37.5	37.8	42.2	46.3	46.6	46.0
Sodium, ≥2.3 grams	73.2	73.8	59.2	59.2	72.9	76.3	76.6	71.5
Potassium, <4.7 grams	97.4	99.3	98.6	99.3	95.0	96.6	95.7	97.9

Key point

People with CKD (defined as either a low eGFR or albuminuria) remained more sedentary than those without, even after accounting for the older age of people with CKD.

Additional context

Sedentary behavior declined among people with CKD, from 33.6% in 2009-2012 to 26.2% in 2017-March 2020, accompanied by a smaller decline among those without CKD (from 24.0% to 20.8%). The percentage currently smoking declined among those with and without CKD, but the percentage who smoked was still slightly higher in those with CKD. Although far less common than smoking, vaping was also more common among people with CKD.

People with CKD in the most recent survey period were slightly less likely to report caloric intake ≥2,000 kcal/day than those without CKD. Regardless of CKD status, a minority of people meet dietary recommendations for sodium or potassium intake (U.S. Department of Agriculture and U.S. Department of Health and Human Services, 2020). The percentage with ≥2.3g per day of sodium intake was lower among people with CKD than among those without CKD.

Summary

The overall percentage of U.S. adults with CKD was 14.0%, based on estimates from the NHANES in 2017-March 2020 (although note that this is based on a single examination demonstrating low eGFR or albuminuria). Although this percentage is only slightly higher than it was in 2005-2008 (13.3%) and virtually identical to the percentage in 2013-2016 (13.9%), this finding may mask an effective decline in the prevalence of CKD because the U.S. population has aged over this period. A stable rate can therefore be considered a form of progress. Evidence for this phenomenon can be appreciated by examining age-stratified results: the percentage of people under age 65 years with CKD increased modestly, from 8.4% to 9.0%, whereas there was a large decline, from 38.0% to 33.2%, among people aged ≥65 years. The reduction in CKD appears to be driven mainly by fewer people across the age spectrum with stage 3 CKD, rather than by a reduction in the percentage of

people with albuminuria. eGFR decline among older people may have slowed over time, but the number of people over age 65 who are at risk of CKD has increased simultaneously. The net result appears to be a stable overall prevalence of CKD in the setting of an aging population.

Prevalence of CKD in general, and the distribution of CKD stage in specific, is worthy of note. Hispanic people have roughly twice the incidence of ESRD compared with White people (Figure 1.2 of the ESRD Volume). A slight decrease in the percentage with CKD 3 or 4 between 2013-2016 was partially offset by a slight increase in the percentage with stage 5 CKD. Given that the overall incidence of ESRD is almost twice as high in this group as among White people, it is possible that this apparent shift, if sustained, could indicate that progression of CKD occurs more quickly in people who are not Hispanic, relative to others.

Increasing rates of obesity, hypertension, and diabetes in the population threatens public health efforts at reducing the prevalence of CKD in the population as a whole and highlights the importance of addressing these major CKD risk factors in efforts to reduce the overall burden of CKD. The need for prevention of risk factors for CKD can be appreciated by considering lifestyle factors. There has been some reduction in sedentary behavior over time, but more than one quarter of those with CKD remained sedentary in 2017-March 2020. Adherence to a low sodium diet was poor, with more than three quarters not meeting recommended targets. These modifiable factors may be contributing to the increase in obesity, diabetes, and hypertension and, ultimately, CKD. Indeed, the prevalence of diabetes has increased in the overall population and especially among people with CKD. The public health risk of sedentary behavior and poor diet are widely appreciated, but there has been less emphasis on their role in the development of CKD than on their impact on cardiovascular disease.

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Detailed Methods

Table 1.1

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We estimated the percentage and number of US adults in the KDIGO CKD risk categories using survey sample weights.

KDIGO CKD Risk Groups Definition:

- *Low Risk*: eGFR ≥60 mL/min/1.73m² and ACR <30 mg/g;
- *Moderately High Risk*: eGFR 45-59 mL/min/1.73m², or the combination of eGFR ≥60 mL/min/1.73m² and ACR 30-299 mg/g;
- *High Risk*: eGFR 30-44 mL/min/1.73m², or the combination of eGFR 45-59 mL/min/1.73m² and ACR 30-299 mg/g, or the

combination of eGFR ≥60 mL/min/1.73m² and ACR ≥300 mg/g;

- *Very High Risk*: eGFR <30 mL/min/1.73m², or the combination of eGFR 30-44 mL/min/1.73m² and ACR 30-299 mg/g, or the combination of eGFR <60 mL/min/1.73m² and ACR ≥300 mg/g.

Abbreviations: ACR, urinary albumin to creatinine ratio; CKD, chronic kidney disease; eGFR, estimated glomerular filtration rate; KDIGO, Kidney Disease: Improving Global Outcomes; NHANES, National Health and Nutrition Examination Survey.

Figure 1.1

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We estimated the percentage of CKD in US adults by demographics and the following comorbidities:

- Diabetes: affirmative self-report, medication for diabetes, or glycosylated hemoglobin ≥7%
- Hypertension: anti-hypertensive medication, systolic blood pressure ≥130 mm Hg, or diastolic blood pressure ≥80 mm Hg
- CVD: myocardial infarction, congestive heart failure, or stroke
- Obesity: BMI ≥30 kg/m²

Abbreviations: ACR, urinary albumin to creatinine ratio; BMI, body mass index; CKD, chronic kidney disease; CVD, cardiovascular disease; NHANES, National Health and Nutrition Examination Survey.

Figure 1.2

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We reported distribution of CKD status by demographics and the following comorbidities:

- Diabetes: affirmative self-report, medication for diabetes, or glycosylated hemoglobin ≥7%
- Hypertension: anti-hypertensive medication, systolic blood pressure ≥130 mm Hg, or diastolic blood pressure ≥80 mm Hg
- CVD: myocardial infarction, congestive heart failure, or stroke
- Obesity: BMI ≥30 kg/m²

Abbreviations: ACR, urinary albumin to creatinine ratio; BMI, body mass index; CKD, chronic kidney disease; CVD, cardiovascular disease; NHANES, National Health and Nutrition Examination Survey.

Figure 1.3

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We estimated percentage of CKD stages in US adults and defined CKD stages as below:

- G3: eGFR 30-59 mL/min/1.73 m²
- G4: eGFR 15-29 mL/min/1.73 m²
- G5: eGFR <15 mL/min/1.73 m²

Abbreviations: ACR, urinary albumin to creatinine ratio; CKD, chronic kidney disease; NHANES, National Health and Nutrition Examination Survey.

Figure 1.4

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We reported the percentage with levels of urinary albumin to creatinine ratio as follows: <10 mg/g, 10-29 mg/g, and 30-299 mg/g.

Abbreviations: ACR, albumin to creatinine ratio; NHANES, National Health and Nutrition Examination Survey.

Figure 1.5

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis:

- We reported percentage of levels of ACR ≥30 by eGFR categories
- The eGFR<30 mL/min/1.73m^2 category is suppressed because the sample size was <30 in at least one set of survey years

Abbreviations: ACR, albumin to creatinine ratio; eGFR, estimated glomerular filtration rate; NHANES, National Health and Nutrition Examination Survey.

Figure 1.6

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We reported the percentage with diabetes among US adults by CKD status; diabetes is defined by affirmative self-report, medication for diabetes, or glycosylated hemoglobin ≥7%. CKD status is categorized by stages:

- G1: eGFR ≥90 mL/min/1.73 m² and ACR ≥30 mg/g
- G2: eGFR 60-89 mL/min/1.73 m² and ACR ≥30 mg/g
- G3: eGFR 30-59 mL/min/1.73 m²
- G4: eGFR 15-29 mL/min/1.73 m²
- G4: eGFR <15 mL/min/1.73 m²

Abbreviations: ACR, albumin to creatinine ratio; CKD, chronic kidney disease; DM, diabetes mellitus; eGFR, estimated glomerular filtration rate; NHANES, National Health and Nutrition Examination Survey.

Table 1.2

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis: We reported prevalence of CKD by insurance status and type, income, and education in US adults overall and among people aged ≥65 years, respectively.

Abbreviations: ACR, albumin to creatinine ratio; CKD, chronic kidney disease; NHANES, National Health and Nutrition Examination Survey.

Table 1.3

Data Source: National Health and Nutrition Examination Survey (NHANES).

Study Population: Participants aged ≥20 years with serum creatinine and urinary ACR measurements.

Analysis:

- We reported the percentage of US adults with different health risk behaviors, both adjusted and unadjusted
- Sedentary behavior was defined as having <2.5 MET-hours per week
- Vaping was defined as affirmative self-report of having vaped during the past 5 days
- We adjusted the percentages to standardize to the age, sex, and race/ethnicity distribution of the population for 2017-March 2020.

Abbreviations: ACR, urinary albumin to creatinine ratio; CKD, chronic kidney disease, eGFR, estimated glomerular filtration rate; MET, metabolic equivalent of a task; NHANES, National Health and Nutrition Examination Survey.